Head-to-Toe Script v3.2 (fast)

"Quotes" = say it · *[brackets] = do it* · **Green = say findings** · Pt = patient reply. Based on the 100% script (M.F. Fabiana). Bring: stethoscope, penlight, watch w/ second hand, sanitizer, alcohol wipes, gloves (mouth only), tongue depressor, Rosenbaum chart, cotton ball, cotton swab.

1. Introduction / Safety

[Knock]

"Can I come in?" Pt: "Yes."

[Hand hygiene + clean stethoscope while introducing]

"Hi, my name is Luke, and I will be your student nurse for today. I am working with your primary nurse. How are you doing?" Pt: "Good."

"Do you have any allergies?" Pt: "No."

"The patient denies any allergies."

"I will be performing a head-to-toe assessment on you today. This means I'll be inspecting, palpating, percussing, and auscultating different areas of your body during this exam. Is it okay if I proceed with this assessment?" Pt: "Yes."

"Okay. If at any point you have any questions or concerns, please let me know."

2. Mental Status

"Do you mind if I take a look at your wristband?" *[check]*

"Can you please state your full name and date of birth for me?"

"Can you please tell me where you are?"

"Can you please tell me what day of the week/year it is?"

"Patient is alert and oriented to person, place, and time."

"Are there any cultural needs I should be aware of for your care today?" Pt: "No."

"The patient denies any cultural needs."

"Are there any spiritual preferences you would like me to be aware of?" Pt: "No."

"The patient denies any spiritual preferences."

3. Head

"First, I will start by inspecting your head." *[look front & back, exaggerate]*

"Upon inspection, the skull is normocephalic, symmetric, and proportionate to the rest of the body."

"Next, I'm going to palpate the skull, please let me know if you feel any pain or tenderness." *[both hands, front to back]* "Did you feel any pain or tenderness?"

"The skull feels smooth to touch. The patient reports no pain or tenderness. There are no abnormal protrusions."

"Before I proceed to the next area, normally I would assess everything bilaterally. But for the sake of time, I will only be assessing one eye, one ear, and one nostril."

4. Eyes

"First, I will begin assessing the eyes."

Visual acuity

"Do you have corrective lenses?" Pt: "Yes, I wear glasses."

"Okay. I'm going to have you take a visual acuity test with a Rosenbaum chart." *[hand chart]*

"If you could please hold this chart with your arm out about 14 inches away from your face while covering your right eye with this card."

"Perfect. Can you please read the lowest line possible?"

"Patient's visual acuity is 20/20 in the left eye while wearing corrective glasses."

PERRLA

"Now I will be inspecting PERRLA. PERRLA stands for pupils are equal, round, reactive to light, and accommodation." *[penlight close, look]*

"The pupils are equal and round. The size is 3mm, which is within the normal range of 3-5mm."

"Can you please look at the wall behind me? I will be shining my penlight into your left eye twice to check for a direct and consensual reaction." *[brief flash, watch left]*

"The left pupil constricts. Therefore, direct light response is present."

"I am going to shine my penlight for a second time to check for a consensual reaction." *[flash left, watch RIGHT]*

"The right pupil constricts. Therefore, consensual light response is present."

"Next, I will be assessing for accommodation."

"I will move my penlight towards you and hold it about 3 inches away from your nose. When I say 'now,' I want you to look at the tip of my penlight. Okay?" *[point at tip]*

"Can you please look at the wall behind me again?" *[bring penlight to 3 in]* "Now."

"The pupils constrict and converge. Therefore, accommodation is present. PERRLA is intact."

5. Ears

"I will now assess the external auditory meatus." *[look, exaggerate]*

"Upon inspection, the ears are bilaterally symmetrical. The skin is intact. No swelling, inflammation, or lesions."

"I will begin inspecting the external auditory meatus using my penlight." *[gently pull pinna up and back to straighten canal, then shine penlight]*

"External auditory meatus is free of swelling, redness, and discharge with some cerumen which is normal."

"Now I'm going to palpate the tragus, pinna, and mastoid process. Please tell me if you feel any pain or tenderness."

"I'm going to pull up the pinna, push down the tragus, and palpate the mastoid process." *[do each as you say it]* "Did you feel any pain or tenderness?"

"Upon palpation of the ear, no pain or tenderness was noted or reported."

6. Nose

"I will now be inspecting your nose." *[inspect only, no touching; exaggerate looking]*

"The nose is symmetrical, midline, and proportional to other facial features. No lesions, deviations, or deformities are present."

"I will now assess the patency of your nostrils."

"Can you please cover and press down one side of your nostril and sniff in and out for me?"

"Patient was able to sniff in and out. Therefore, there is no obstruction in the nares. Therefore, the patient has full patency."

7. Mouth

"Next, I will begin inspecting your mouth, which includes your lips, gums, teeth, and tongue." *[gloves on, tongue depressor ready]*

"First, I will begin inspecting your outer lips."

"Upon inspection, the outer lips are pink and moist with no cracking and lesions present."

"Using a tongue depressor, I am going to retract the lips to inspect the inner lips and gums." *[lift upper, push lower; check gums at same time]*

"Inner lips are pink, moist, and free of lesions and fissures as well."

"Both upper and lower gums are pink with a stippled surface. The gum margins at the teeth are tight and well-defined. No bleeding, retraction, or edema is present."

"Let's assess the teeth next." *[penlight + depressor]*

"The teeth look white, clean, straight, and evenly spaced with no caries, missing, or loose teeth."

"Can you please stick your tongue out for me?" *[depressor slightly off center]*

"The color of the tongue is pink. The dorsal surface is moist and roughened with papillae. No nodules and ulcerations are present."

"Can you please lift your tongue to the roof of your mouth?"

"The ventral surface is smooth and glistening. No nodules and ulcerations. Some veins are present."

8. Motor System

"Next, I will be assessing the motor system."

"I will start by inspecting your gait."

"Please walk in a straight line toward this wall and on your way back, walk heel-to-toe, one foot in front of the other."

"Gait is smooth, rhythmic, and effortless. The tandem walk was straight, balanced, and coordinated."

"I will now assess your strength with a grip test. Please squeeze my fingers." *[cross index + middle fingers, both hands]*

"The grip strength is strong and symmetric bilaterally."

9. Sensory System

"I will now assess the sensory system by testing light touch and pain."

"I will begin with the light touch test. Please close your eyes and say 'now' when you feel the cotton ball." *[face: forehead, cheek, jaw; then arms, forearms, hands, chest, thighs, legs, random order]*

"The patient was able to correctly identify light touch sensations in all tested areas. Therefore, light touch is intact."

"For the pain test, this will be sharp, and this will be dull. Please close your eyes and tell me whether you feel a sharp or dull sensation." *[same periphery areas, random]*

"The patient was able to accurately and correctly distinguish between sharp and dull sensations. Therefore, pain sensation is intact."

10. Skin

"I will now do a skin assessment."

"I will start by inspecting the skin." *[look, exaggerate]*

"Upon inspection, the patient's skin tone is light tan, homogenous, and consistent with genetic background."

"I will palpate for temperature and moisture." *[back of both hands: forehead → cheeks → neck → shoulders → arms → hands → thighs → legs → toes]*

"Upon palpation, the patient's skin temperature is warm and equal bilaterally with no diaphoresis and dehydration."

"Upon palpation, there is no excessive moisture or sweating."

"Now I will assess for edema. I will be firmly pressing my thumbs down into your tibia for 3 to 4 seconds." *[look for dent]*

"No pitting edema noted bilaterally in the lower extremities."

11. Thorax & Lungs (posterior only)

"Next, I will assess the thorax and lungs."

"I will first inspect the shape and configuration of your posterior chest. But before I do that, I will have you swing your leg over to the opposite side of the bed." *[look at back; AP-to-T hand motions]*

"Upon inspection, I can see that the anteroposterior diameter is less than the transverse diameter. Therefore, the AP to T ratio is 1:2. No barrel chest is present. The spinous processes are straight. The thorax is symmetric, in an elliptical shape with ribs sloping downwards at a 45-degree angle. Scapulae are placed symmetrically in each hemithorax."

"Now I will auscultate for breath sounds."

"Please lean forward slightly, hands on lap. Deeply breathe in and out through your mouth every time you feel my stethoscope touch your back. Please let me know if you want to take a break or if you are feeling dizzy or lightheaded." *[spots 1–7 on back, 8–9 on sides]*

"Vesicular sounds were heard in the peripheral lung fields."

"Bronchovesicular sounds were heard over the major bronchi."

"Breath sounds are clear and equal bilaterally with no adventitious sounds like crackles or wheezing."

12. Peripheral Vascular

"I will now assess the peripheral vascular system. Can I have you lie down, please?"

"I will begin by palpating for your pulse."

"I am now palpating the carotid pulse on each side." *[ONE side at a time]*

"I am palpating the radial pulses."

"I am palpating the dorsalis pedis pulses."

"I am palpating the posterior tibial pulses."

"All pulses in assessed locations are 2+ bilaterally. Rate is normal and rhythm is regular and equal bilaterally."

"I am now going to check for capillary refill at heart level." *[fingertip at heart level; press nail edge at least 5 sec; release]*

"Capillary refill is normal because color returns in less than 2 seconds."

★ Bonus: pulse grading scale

"The pulse is graded on a 3-point scale: 3+, increased, full, bounding. 2+, normal. 1+, weak. 0, absent."

13. Heart

"I will now assess the heart."

"First, I will auscultate over the 5 cardiac areas with my diaphragm."

"I am now auscultating the aortic valve area." · "I am auscultating the pulmonic valve area." · "And now the Erb's point." · "Then the tricuspid valve area." · "Finally, the mitral valve area."

"S1 and S2 sounds were heard in all areas. Rhythm is regular and even."

"I will now auscultate in reverse order with my bell for S3 or S4 sounds or heart murmurs." *[mitral → tricuspid → Erb's → pulmonic → aortic]*

"No S3, S4, or heart murmurs were heard during auscultation."

14. Abdomen

"I will now assess the abdomen. Before I start, is there any tenderness or pain that I should be aware of?"

"I will now inspect the abdominal contour." *[penlight; look from above, then eye level]*

"The contour is flat, smooth, and symmetrical with no visible masses, bulges, or distention."

"Now I will auscultate for bowel sounds over the right lower quadrant with the diaphragm of my stethoscope." *[start RLQ; listen until first sound. Heard → stop. None → next quadrant clockwise]*

"Bowel sounds are present, high-pitched, active, and gurgling over the right lower quadrant. If no bowel sounds were present, I would listen in all 4 quadrants for 5 minutes before noting no bowel sounds present."

"Next, I will percuss the abdomen in each quadrant, starting at the right lower quadrant." *[ONLY middle finger of non-dominant hand on skin; tap it with dominant middle finger]*

"Tympany was heard throughout all quadrants of the abdomen."

"Lastly, I will perform a light palpation of the abdomen. I will use my first 4 fingers to palpate all 4 quadrants of the abdomen about 1cm deep in a circular motion and zig-zag pattern. Please let me know if you feel any pain or tenderness." *[4 fingers flat, all 4 quadrants]*

"The abdominal tone is smooth and soft with no muscle guarding and no reported tenderness."

[Stop: did I do EVERYTHING?]

"This concludes my head-to-toe assessment. Thank you very much!"